INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Fruquintinib (Fruzaqla) monotherapy

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
28 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00890
Version 2a

21 days of capsules, then seven days without capsules

Treatment stages

01 **21 capsule days, then seven rest days**
28-day cycle

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Get urgent help for severe or persistent tummy pain, black or bloody stools, coughing or vomiting blood, new severe headache, confusion, seizure, visual change, chest pain, sudden breathlessness or very little urine. Report fever, sudden illness, painful hands/feet and new bleeding promptly.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

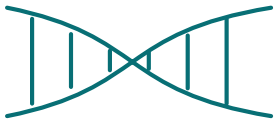
Your targeted medicine

Fruquintinib is a targeted capsule treatment to help control bowel cancer that has spread after previous treatments. The team checks which treatments you have received, whether they remain suitable, and your priorities.

TARGETED THERAPY CAPSULE

Fruquintinib

Fruzaqla



Blocks VEGF receptor signals

Fruquintinib blocks VEGF receptor signals that help tumours form blood vessels. Reducing these signals can slow cancer growth. Blood vessels in healthy tissues are also affected, which helps explain blood-pressure, bleeding, protein-in-urine and wound-healing risks.

VEGF signals → Receptors blocked → Blood-vessel growth reduced

What to watch for

Report painful hands/feet, bowel changes, bleeding or neurological symptoms; attend blood-pressure, urine and blood checks even if well.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Why this treatment may be offered

Confirm the treatment history and personal goals.

Previously treated bowel cancer

Fruquintinib is a targeted capsule treatment to help control bowel cancer that has spread after previous treatments. The team checks which treatments you have received, whether they remain suitable, and your priorities. The team confirms the full treatment history, including chemotherapy, anti-VEGF and anti-EGFR treatment where applicable, and progression or intolerance on trifluridine/tipiracil or regorafenib.

Evidence and uncertainty

Median overall survival was 7.4 months with fruquintinib and 4.8 months with placebo. These group medians are not a prediction of one person’s lifespan or a guaranteed extra 2.6 months. The team should discuss the balance of benefit and side effects for you.

Treatment duration

There is no fixed total number of cycles. Continue while benefit and tolerability justify it; the team reviews symptoms, scans, tests and your preferences.

How fruquintinib works

It affects signals involved in blood-vessel growth.

Safety checks

Blood-pressure and urine-protein changes can develop without symptoms. Tests and early symptom reporting help the team decide whether treatment can safely continue.

Evidence in previously treated colorectal cancer

FRESCO-2 compared fruquintinib plus best supportive care with placebo plus best supportive care after previous metastatic colorectal treatments.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Capsules and rest days

Fruquintinib (Fruzaqla) monotherapy

28-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

28-DAY CYCLE

21 capsule days, then seven rest days

D1–21 D22–28

Take the prescribed fruquintinib dose once daily on Days 1–21, then take no fruquintinib on Days 22–28. Take it at about the same time each day, with or without food. Repeat only when your team authorises the next cycle.

No fixed final cycle. Continue only while benefit and tolerability justify treatment.

BASELINE → TREATMENT → FOLLOW-UP

Tests before, during and after treatment

NCCP specifies regular blood/urine/thyroid tests but no universal frequency for each. Ask your team to write the dates. Scans and tumour markers are individualised.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00890 v2a official fruquintinib regimen

Tests in your plan

Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN

For a colorectal cancer plan, if your team uses CEA; your team sets the interval.

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

Fruquintinib (Fruzaqla) monotherapy: treatment and checks

Metastatic colorectal cancer after standard therapies and trifluridine/tipiracil or regorafenib

Take the prescribed fruquintinib dose once daily on Days 1–21, then take no fruquintinib on Days 22–28. Take it at about the same time each day, with or without food. Repeat only when your team authorises the next cycle.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First month	Later cycles	A hold or procedure	After treatment
Blood pressure NCCP 00890	Control existing hypertension	Weekly blood-pressure checks	Monthly and as clinically indicated	Review control before restarting	Follow persistent hypertension as agreed
Blood count, kidneys, liver and thyroid NCCP 00890	Baseline tests	Regular tests on the team plan	Continue regular tests	Repeat abnormal tests as directed	Follow unresolved effects
Urine protein NCCP 00890	Baseline urine dipstick	Regular urine checks	Extra measurement if protein is detected	Review kidney/urine findings before restarting	Individual follow-up if abnormal
international normalised ratio (blood-clotting test) (INR) if taking warfarin NCCP 00890	Review anticoagulation	Weekly INR until a stable dose	Then before each cycle	Additional checks when medicines or health change	Anticoagulation-team plan
Scans and clinical review NCCP 00890	Confirm previous treatments and goals	Review symptoms, skin, nutrition and adherence	Disease assessment as directed by consultant	Team authorises any restart	Agree subsequent care and contacts

What each check is for

1 21 capsule days, then seven rest days

28-day cycle

Fruquintinib is a targeted capsule treatment to help control bowel cancer that has spread after previous treatments. The team checks which treatments you have received, whether they remain suitable, and your priorities.

Ask / act: Confirm the current phase, prescription, tests and support medicines.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Prescription and calendar

STEP 2

Taking capsules

The usual starting dose is 5 mg once daily, but follow your own label. Swallow capsules whole with water, with or without food; do not chew, dissolve or open them. Tell the team before starting if swallowing capsules is difficult.

STEP 3

Missed or vomited dose

If a dose is less than 12 hours late, take it and then follow the usual schedule. If more than 12 hours late, skip it and take the next dose at the normal time. At exactly 12 hours, or if unsure, ask your pharmacist or team. Do not take a replacement capsule after vomiting; take the next dose at the usual time.

STEP 4

Tests and visits

Baseline and regular blood count, kidney/liver profile, thyroid function and urine protein. Blood pressure must be controlled before starting, then checked weekly for the first month and monthly thereafter, or more often if needed. For warfarin, INR weekly until stable, then before each cycle. Scans and markers follow the consultant plan.

STEP 5

Storage

Keep the original bottle tightly closed to protect from moisture and out of children’s sight and reach. No special temperature conditions are required. Follow the expiry date and ask the pharmacist how to return unused capsules; do not put them into household waste or wastewater.

STEP 6

Treatment changes

Your team must confirm dose changes and restarts. Bring the capsule pack, current calendar and medicine list to review.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Your 28-day cycle.

Timing is a general guide; symptoms may start or stop at other times.

Treatment or check	Timing in this view
Fruquintinib capsules	 **Days 1–21** Once daily, with or without food
Capsule-free week	 **Days 22–28** No fruquintinib; review recovery and next prescription
Symptoms, blood pressure (BP) and tests	 **Days 1–28** Report symptoms on treatment days and rest days

The calendar shows capsule days and rest days. Symptoms are not confined to a particular week.

The next cycle starts 28 days after this one, if your team confirms it.

> Use your oncology unit's urgent contact plan throughout the cycle, including tablet-free days. Symptoms can need attention before the next blood test or appointment.

Between-treatment notes

Day 1–7: Start with a clear calendar

Take the prescribed daily dose with or without food. Keep blood-pressure appointments and check hands and feet. Confirm the first tests, urine checks and skin-care instructions.

Day 8–21: Continue capsules and monitoring

Symptoms may develop over time. Report painful hands/feet, bowel changes, poor appetite, bleeding or fatigue. Blood pressure is checked weekly during the first month. Do not wait until the rest week to report problems.

Day 22–28: Capsule-free week

Take no fruquintinib. Symptoms may persist. The team may delay the next cycle or prescribe a lower dose. Complete planned checks and confirm the next dose and start date.

Support around the medicines

1 Skin care

Use the unit’s skin-care plan and report redness, pain, swelling or blisters early. The team may prescribe creams or change treatment. Do not wait until walking or everyday hand use is difficult.

2 Sickness and bowels

This oral medicine has minimal-to-low emetogenic potential. Routine premedication is not usually needed; agree any anti-sickness and diarrhoea medicines with the team. Persistent vomiting, diarrhoea, poor intake or reduced urine needs prompt advice.

3 Blood pressure and medication review

Take prescribed blood-pressure medicines and keep monitoring visits. Tell the team about anticoagulants and every prescription, over-the-counter and herbal medicine.

4 Pregnancy and breastfeeding

Use highly effective contraception during treatment and for at least two weeks after the last dose if you could become pregnant. Do not breastfeed during treatment or for at least two weeks afterwards.

Your capsule and rest-day plan

Use your own prescription and calendar.

If too much is taken

Contact your doctor immediately if you take more than prescribed. You may need medical attention; take the pack and treatment calendar with you.

Checks and follow-up

Feeling well does not replace testing.

If a test is abnormal

The team may arrange extra blood or urine tests, treat hypertension, pause capsules, reduce the dose or stop treatment. Do not change the dose yourself.

Scans and tumour markers

The consultant decides the imaging and marker schedule for your situation. This regimen does not mandate a fixed computed tomography (CT) or carcinoembryonic antigen (CEA) interval.

After a pause or stopping

Ask who follows persistent symptoms and abnormal tests, and when the next review is due. Do not restart without clear instructions.

Skin, mouth, eating and energy

Early support can help preserve daily activities.

Mouth and appetite

Report mouth soreness, voice changes, poor appetite or weight loss. Ask for mouth care and nutritional support. Drink as advised for your health.

Fatigue and thyroid

New marked fatigue, feeling unusually cold, weight or skin changes may need thyroid assessment. Balance activity with rest and report symptoms affecting usual activities.

Driving

Do not drive or use machinery if tiredness or other symptoms affect concentration or reactions.

Symptoms needing urgent help

A rare effect may still need immediate care.

Bleeding and bowel problems

Seek urgent help for black or bloody stools, blood in urine, coughing/vomiting blood, severe or persistent abdominal pain. Heavy bleeding or collapse needs emergency care.

Neurological, heart or vascular symptoms

Severe headache with confusion, seizure or visual changes, chest pain, severe breathlessness, sudden weakness or sudden severe chest/back/abdominal pain requires emergency help.

Kidneys, skin and infection

Report much less urine or swelling, painful hands/feet, fever, shivering or suddenly feeling unwell. Symptoms may need assessment even during the capsule-free week.

Other medicines, wounds and procedures

Tell every treating team you take fruquintinib.

Medication review

Tell the pharmacist about anticoagulants, antibiotics, infection treatments, seizure medicines, herbal remedies and supplements. Rifampicin and other strong enzyme inducers can affect fruquintinib exposure. Ask before starting or stopping anything.

Wound healing and surgery

Report unhealed wounds and planned surgery. Product information recommends withholding fruquintinib for at least two weeks before surgery and not restarting for at least two weeks afterwards and until adequate wound healing. Your oncology and surgical teams must agree the exact plan; do not stop or restart it yourself.

Allergies

Tell the pharmacist about medicine or colouring-agent allergies. Capsule colours differ by strength and contain colouring agents that can cause allergic reactions.

Pregnancy precautions and safe storage

Keep the medicine in its labelled packaging.

Your treatment record

Carry the wallet card and current medicine list. Record the prescribed dose, any holds, symptoms, BP and upcoming tests. Replace all example telephone numbers with your real oncology contacts before using the card.

Questions worth asking

undefined

undefined

undefined

undefined

undefined

undefined

undefined

undefined

undefined

undefined

undefined

undefined

Try explaining the plan back: Can you show the 21 tablet days and seven rest days, explain the meal and missed-dose instructions, identify the early liver and blood-pressure checks, and say whom you will call if unwell?

Previously treated colorectal cancer The team confirms the full treatment history, including chemotherapy, anti-VEGF and anti-EGFR treatment where applicable, and progression or intolerance on trifluridine/tipiracil or regorafenib.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Fruquintinib: Days 1–21, then Days 22–28 off

Active medicines: Fruquintinib

Emetogenic risk: Minimal-to-low oral

Medicine	Dose / route	When
Rescue antiemetic	Individual prescription under the oral-systemic anticancer therapy (SACT) pathway	As required; no routine daily prophylaxis NCCP classification V6 Table 5 provides metoclopramide, prochlorperazine or one 5-HT3 antagonist as options; choose rather than automatically combine, after interaction/contraindication review.

Additional phase-specific support

- Premedication is not usually required. Use local skin-care guidance for hand-foot skin reactions and report painful, blistered or function-limiting symptoms early.
- Blood pressure weekly for the first month, then monthly and as clinically indicated. Keep prescribed antihypertensive treatment and report severe headache, visual change, chest pain or neurological symptoms urgently.
- Baseline and regular full blood count (FBC), renal/liver profile, urine dipstick for protein and thyroid function. international normalised ratio (blood-clotting test) (INR) if clinically indicated; if taking warfarin, weekly INR until a stable warfarin dose is established, then before each cycle.
- Review bleeding, gastrointestinal perforation, wounds and planned surgery; follow the separately prescribed interruption/restart plan. Do not borrow the regorafenib dose-modification table.
- Women of childbearing potential: highly effective contraception during treatment and for at least two weeks after the last dose, per NCCP.
- Confirm the prescribed daily dose and Days 1–21 calendar; dose reductions are consultant-led and treatment must stop if 3 mg daily is not tolerated.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Head / thyroid / energy

Fruquintinib

Fatigue and thyroid changes can affect daily life. Severe headache, confusion, seizure or visual change needs emergency help.

3 Heart / blood pressure

Fruquintinib

Blood pressure may rise without symptoms. Keep scheduled checks. Chest pain, collapse or severe breathlessness needs emergency care.

5 Liver

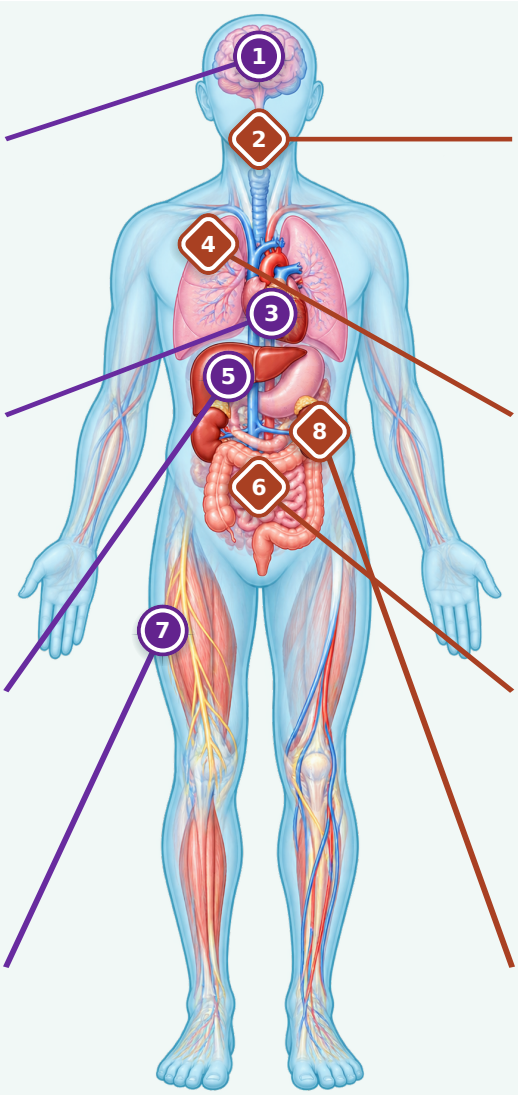
Fruquintinib

Liver tests can change before symptoms occur. Report yellow eyes or skin, dark urine or persistent abdominal pain.

7 Hands / feet / skin

Fruquintinib

Pain, redness, swelling or blisters can affect walking or using your hands. Report early and follow the skin-care plan.



2 Mouth / appetite

Fruquintinib

Mouth soreness, voice change and poor appetite can affect eating. Report difficulty drinking or weight loss.

4 Blood / bleeding

Fruquintinib

New bleeding, easy bruising, fever or sudden illness needs prompt advice. Heavy bleeding needs emergency help.

6 Bowel / abdomen

Fruquintinib

Diarrhoea can cause dehydration. Severe persistent pain, black stools or blood in stools needs urgent help.

8 Kidneys / urine

Fruquintinib

Protein in urine may have no early symptoms. Attend urine checks and report swelling or much less urine.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Get urgent help for severe or persistent tummy pain, black or bloody stools, coughing or vomiting blood, new severe headache, confusion, seizure, visual change, chest pain, sudden breathlessness or very little urine. Report fever, sudden illness, painful hands/feet and new bleeding promptly.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

Record each cycle separately. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Use a new record sheet for each cycle or review. Copy or print another sheet when needed; the number of sheets does not prescribe the length of treatment.

Cycle number: _____ Start date: _____ Next review: _____

Record the prescribed dose, treatment days, rest days, blood-pressure readings and tests. A diary entry is not permission to restart after a hold.

Fruquintinib · Targeted therapy capsule

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Hand-foot skin reaction	□ None	□ Redness or tingling without pain	□ Pain limits usual tasks: call	□ Pain limits washing / dressing: urgent review	□ Acute severe illness: emergency help
Diarrhoea	□ Usual pattern	□ Up to 3 extra stools above normal	□ 4–6 extra stools/ day or at night: call	□ 7+ extra stools/ day or dehydration: urgent review	□ Collapse / severe dehydration: 112/999
Mouth soreness / ulcers	□ None	□ Mild soreness; eating normally	□ Painful; soft food needed: call	□ Cannot eat / drink enough: urgent review	□ Major breathing difficulty: 112/999
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Keep your treatment details to hand

Metastatic colorectal cancer after standard therapies and trifluridine/tipiracil or regorafenib · NCCP 00890 v2a · Document revised 5 November 2026

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

Fruquintinib (Fruzaqla)

28 days · capsules Days 1–21 · no capsules Days 22–28.

Phase: _____ Cycle plan: team to confirm

Daytime (EXAMPLE) 087 12345678

Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

CUT outer border

BACK / TREATMENT RECORD

My cycle record

Rolling record — follow your agreed treatment plan.

Cycle	Date / initials

My treatment information

Latest treatment date: _____

Plan changed? Ask the team to update this card.

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Hospital / team: _____
Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

SACTCheck v0.84.35 · NCCP 00890 v2a · Wallet-card sheet
Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

CHAPTER 05

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00890 v2a official fruquintinib regimen**
   https://healthservice.hse.ie/documents/6648/Fruquintinib_890.pdf
2. **EMA Fruzaqla product information and patient leaflet**
   https://www.ema.europa.eu/en/documents/product-information/fruzaqla-epar-product-information_en.pdf
3. **FRESCO-2 randomised phase III trial**
   https://pubmed.ncbi.nlm.nih.gov/37331369/
4. **HSE treatment safety and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
5. **NCI: colon cancer treatment — carcinoembryonic antigen (CEA) and follow-up**
   https://www.cancer.gov/types/colorectal/hp/colon-treatment-pdq
6. **NCCP regimen; NCIS Medical Oncology antiemetics V8 Table 1**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf